A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Keryn Walters
West Brunswick Clinic

Dear Dr Walters,

Re: Mrs Rachel Huelsebusch-Smith
DOB: 1976-02-11
Address: 24 Wheeler Street, PASCOE VALE SOUTH VIC 3044

Summary

- **Active eosinophilic oesophagitis confirmed on histopathology.**
- **Minor colonic polyp removed with no further follow-up required.**
- **Trial of intermittent proton pump inhibitor therapy for symptom management.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Rachel following her recent gastroscopy and colonoscopy.

The colonoscopy was largely unremarkable. I removed a very small, insignificant polyp, which requires no further surveillance or concern. However, the gastroscopy revealed subtle endoscopic signs of eosinophilic oesophagitis, including visible furrows. These findings were subsequently confirmed by biopsies, which demonstrated active eosinophilic oesophagitis.

Rachel reports intermittent symptoms of dysphagia, specifically a sensation of food sticking in the oesophagus, which occurs approximately once a month. She noted that certain foods, such as rye bread, often trigger this discomfort. She also mentioned an improvement in a chronic cough that had been present for six months. Notably, she has a known allergy to raw honey, which causes oropharyngeal swelling, and she has observed changes in her tolerance for spicy foods since her pregnancy.

Impression and Plan

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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We discussed the management of eosinophilic oesophagitis in detail, including the three primary treatment pillars: proton pump inhibitors (PPIs), topical steroids (much more effective), and elimination diets. Rachel is currently hesitant to commit to long-term pharmacotherapy. We discussed the possibility of an exclusion diet; however, she noted that a strict six-food elimination diet would be quite restrictive given her high consumption of dairy and eggs.

I have suggested a trial of an intermittent PPI (such as esomeprazole or pantoprazole) for one to two months to determine if this induces symptomatic remission. If successful, she may use this therapy periodically as symptoms arise. I have also advised her on emergency measures for food bolus obstruction, specifically the use of carbonated beverages like Coca-Cola to help dislodge a bolus.

I have reassured Rachel that eosinophilic oesophagitis is not associated with an increased risk of oesophageal malignancy. We will not pursue further endoscopic surveillance at this stage unless her symptoms escalate in frequency or severity, which might suggest the development of an oesophageal stricture. I will provide her with written information regarding her dietary and medical options and have provided a prescription for a PPI. I will review her on an as-needed basis.

Kind regards

HISTOLOGY:

MICROSCOPIC DESCRIPTION And DIAGNOSIS 1. Mid oesophagus - Sections show squamous mucosa with moderate oesophagitis. There are 45 eosinophils per high-power field. **The features are consistent with eosinophilic oesophagitis.** 2. Lower oesophagus - Sections show squamous mucosa with moderate oesophagitis. There are 50 eosinophils per high-power field. 3. Gastric biopsy - Gastric antral mucosa with no erosion or inflammation. No metaplasia or dysplasia. No Helicobacter seen. 4. Duodenal biopsy - Normal villous architecture with no inflammation or erosion. No evidence of coeliac disease or parasites. 5. Random colon biopsies - Normal crypt architecture. No erosion. No inflammatory infiltrate. No thickening of the collagen plate. No atypia. 6. Rectal polyp - A hyperplastic polyp. Pathologist: Dr Melissa Robbie

Yours sincerely,

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